

BCG

OUTPROMPT

THE AI STRATEGY CHALLENGE

PROBLEM 1:

GLP-1 - THE INDIA WEIGHT-LOSS OPPORTUNITY

ROUND 1 SUBMISSION BRIEF

The Situation

You advise a major Indian pharma company (a Sun / Dr. Reddy's / Cipla-type generics major) planning its GLP-1 play as **semaglutide goes off-patent in India in 2026** and a generic wave begins. GLP-1 (anti-obesity / diabetes) is the biggest story in global pharma. But India is largely **out-of-pocket, price-sensitive, and about to be flooded with cheap generics** - so the molecule itself will not be where the money or the moat is for long.

The US is the early signal of where this goes. There, GLP-1 has become a mass consumer phenomenon: roughly **1 in 4 adults is using, has used, or is considering one**, the active base has grown from ~6M in 2022 to ~16M today, and is projected at 30–35M by 2030.

And the most important pattern in the US data is this: **GLP-1 is a behaviour business, not just a molecule**. Users cut calories ~29%, but food habits revert within 1–3 months once they lapse - while 20–40% of *wellness* behaviours (supplements, fitness, tracking) persist. A third of users lapse; about two-thirds of those plan to restart.

Why India Is a Massive Market

India's metabolic burden is enormous and rising. By the ICMR-INDIAB national study (2023), roughly **101 million Indians have diabetes and another 136 million have prediabetes**; about **254 million have generalised obesity and 351 million abdominal obesity**. Adult overweight/obesity prevalence reached ~24% of women and ~23% of men in NFHS-5 (2019-21), up sharply over fifteen years - obesity, once a condition of affluence, is now spreading across income groups and into younger ages. The clinical need is vast; the open question is how much of it converts to a *paying, out-of-pocket* GLP-1 market, and how fast.

Exhibits

Note: most hard data is **USA based**

Exhibit A - Four purchase archetypes forming among active users (each a distinct go-to-market)

	Lifestyle optimization	Disease management	Aesthetic transformation	All-in on GLP-1s
Share of active users	41%	27%	19%	13%
What they buy around the drug	fitness, wellness routines, habit-building	managing the condition & GLP side effects	medical aesthetics, beauty products/services	transforming entire health & wellness life

Exhibit B - Adherence is a flywheel, not a funnel (US users)

Lifecycle fact (US ever-users)	Figure
Active vs lapsed	~66% active / ~34% lapsed
Lapsed users who plan to restart	~60% (about 2/3)
Active users who cycle on/off	12%+
Food habits revert post-lapse	within 1–3 months
Wellness behaviours* that persist post-lapse	20–40%
US adults who have NOT tried a GLP-1	~80%

*Wellness behaviours include taking regular supplements, personal care routine, fitness classes and cardio exercise, health wearables, etc.

Exhibit C - Weight-loss goals vary sharply by country (% of users, by target as share of body weight)

Weight-loss goal	USA (N = 76)	UK (N = 38)	India (N = 78)	Brazil (N = 79)	Mexico (N = 39)
>25%	23%	25%	24%	13%	6%
20–25%	17%	14%	9%	10%	3%
15–19%	20%	17%	12%	17%	17%
10–14%	16%	17%	26%	32%	22%
<10%	23%	28%	28%	28%	53%

(Figures above are directional, low N)

Exhibit D - The molecule is commoditising (pricing & format economics)

Molecule / pricing fact	Figure
US current monthly price	~\$250-400/month
US oral monthly price	~\$150-300/month
US injectable monthly price	~\$300-550/month
US consumer's ideal price point	\$50–90/month
Oral bioavailability vs injectable	~1% vs ~89%; needs 146x the API

Exhibit E - Where US food-spend savings get reallocated. Of money saved on food owing to GLP usage, ~1/3 goes to savings, ~1/3 to the GLP co-pay, and ~1/3 to other spend - mostly supplements, beauty/personal care, apparel, fitness, and aesthetics.

~70% of GLP households changed eating and drinking behaviour, expanding the footprint ~+50% beyond active users (e.g., 16M users ≈ 24M in demand).

The Tension at the Heart of This Case

Post-patent, semaglutide becomes a cheap commodity (net prices are falling even before generics). Competing on price and access alone is a race to the bottom in an out-of-pocket market, despite the size of the market.

Your Decision

Define the India GLP-1 strategy for this client. You may start from these angles and sharpen them:

- **Where to play:** which patient / consumer segments to target first, given India's weight-loss ambition and out-of-pocket reality.
- **How to win beyond the molecule:** what full-stack offer - app, coaching, adherence engine, wellness/aesthetics ecosystem, services - turns a commoditising drug into a durable business.
- **Pricing & access:** how to price and reach an out-of-pocket market, and whether to lead or defend against the generic wave.
- **The 5-year shape:** roughly what patient reach, revenue, and margin trajectory this implies - directional, not a model.

Data You Can Explore

Feel free to use public sources as required – however, ensure you cite them properly so we can go back and verify that these numbers are correct.

What Round 1 Is - and Isn't

Working with AI is part of the job. Modern consulting increasingly expects you to work alongside AI, so that is the skill we are testing here. We are not looking for an answer solved unaided - we want to see how well you direct an AI towards a sharp recommendation, and how well you recognise where it falls short. So please do use AI to develop your response; it is expected, not optional.

Round 1 is analysis, not a build. You are not building a working prototype - that is Round 2. Concept sketches, mock-ups, or described experiences are welcome, but the deliverable is your thinking on the page.

Your Round 1 answer is **not binding**. If Round 2 changes your view, change it with no penalty - the two rounds are scored independently.